



IT'S TUESDAY MORNING. The outpatient clinic in South Delhi is busy. Forty-four-year-old accountant Vikash Solanki is with his endocrinologist. He has one question that has been on top of his mind for a few days now.

Solanki, who weighs 95 kilos

despite his best efforts, came across a post on social media about a generic version of semaglutide, the injectable obesity drug that became a global phenomenon under brand names Ozempic and Wegovy. Unlike those expensive drugs, this one was priced at ₹1,800 a month, at least ten times cheaper than the blockbusters.

Should he take the shot, he asks.

His doctor has been here before. He has been through variations of this conversation in recent weeks. In fact, the frequency has increased since March. He takes a deep breath and begins to explain the pros and cons of the so-called “wonder drug”.

For endocrinologists, this has become the default question since the expiry of patents for semaglutide. This has resulted in one of the sharpest price disruptions for the chronic therapy in recent years.

More than 20 generic manufac-

turers have entered the market in quick succession, according to the National Pharmaceutical Pricing Authority. And prices have fallen by as much as 90%.

There's a rush among Indian companies—including Sun Pharma, Dr Reddy's Laboratories, Zydus Lifesciences and Torrent Pharmaceuticals—who have already launched their generic versions. In fact, semaglutide's innovator, Danish drug maker Novo Nordisk, has also slashed prices on starting doses of its diabetes formulation of semaglutide by 36%, and on its obesity formulation, Wegovy, by 48%.

Importantly, while semaglutide has gone off-patent, Mounjaro is still under patent in India till 2036 and is positioned as a higher-priced, next-generation therapy.

For clinicians, pharmaceutical executives and public health specialists, the harder test is whether

WEIGHTY ISSUES

• About one in four Indian adults is overweight or obese, but a far larger population may be at risk



Overweight/
obese adults
(BMI)

23-24%
(NFHS-5)



Urban
overweight/
obesity

30%+



Rural
overweight/
obesity

15-20%
(low but
rising)



Abdominal
obesity
(women)

~40%



Abdominal
obesity
(men)

~12%



People
with
diabetes

101
MILLION



Pre-
diabetic
population

136
MILLION



Hyperten-
sion
prevalence

~25-30%
ADULTS



Cardiovasc-
ular disease

~28%
OF TOTAL DEATHS



NCD share
of deaths

~63%



Obesity in
women (urban
hotspots)

35%+
IN SOME CITIES



Childhood
overweight/
obesity

~2-4%
(NFHS)

SOURCE ICMR-INDIAB STUDY (2023); NFHS-5 (2019-21); MINISTRY OF HEALTH & FAMILY WELFARE